

AETHERA · SPECIALTY BILLING SERIES

Hematology/Oncology Billing Solutions

Maximizing Revenue Across Office E/M, Chemotherapy Infusion, Radiation
Oncology, Bone Marrow Biopsy, and Clinical Trials.

Prepared by: Aethera Healthcare Solutions · Lakeland, FL

S0100 · 99202-99215 · 96365-96417 · 38100-38240 · 77301-77799 · J-codes
(chemo + supportive)

Updated: 2026 Edition · aetherahealthcare.com · + 1 (863) 694-0325



SPECIALTY BILLI





♦ ABOUT AETHERA

Maximizing Revenue. Minimizing Burden.

Aethera Healthcare Solutions is your full-service medical billing partner headquartered in Lakeland, FL. We handle coding, claims, payments, denials, and collections — so you can focus on patients.

500+

Practices Served
Across 25+ specialties

\$12M+

Revenue Recovered
For specialty practices

95%

Clean Claim Rate
vs 84% industry avg

< 30d

Average AR
Days in receivables



Personal, Not Corporate

Dedicated account team with deep specialty and payer knowledge. Your oncology practice gets a hemonc-trained RCM specialist — not a generic call-center queue.



Performance Targets, In Writing

96%+ net collection · < 5% denial · 95%+ coding accuracy · 24 h posting · 48 h submission · 65%+ appeal success.



Zero Risk To Start

No setup fees, no long-term contracts, 90-day cancellation, free 5-day audit. We earn the relationship through measurable recovery — not contract lock-in.



HEMATOLOGY/ONCOLOGY · OVERVIEW

The Four Pillars of Hematology/Oncology Billing

Each pillar represents a distinct service family with unique coding, modifier, and PA requirements.

01 · OFFICE E/M + CONSULTS



Office E/M & Consultations

Office visits (99202–99215), prolonged services (99354–99357), telehealth follow-ups. Modifier 25 critical when E/M same day as chemo infusion or biopsy.

CPT 99202–99215 · 99354–99357

02 · CHEMO INFUSION



Chemotherapy Infusion & Administration

Chemo IV infusion (96413 first hour, 96415 add'l hour), chemo administration (96405–96411), IV hydration (96360–96361). Pre-auth mandatory. J-codes for chemo drugs.

CPT 96413 · 96415 · 96405–96411 · J-codes · PA mandatory

03 · PROCEDURES & BIOPSIES



Bone Marrow, LP & Paracentesis

Bone marrow biopsy (38221), bone marrow aspiration (38220), lumbar puncture (62270–62272), paracentesis (49080–49081), thoracentesis (32000–32554).

CPT 38221 · 38220 · 62270 · 49080 · 32554

04 · RADIATION + TRIALS



Radiation Oncology & Clinical Trials

IMRT (77301, 77338), stereotactic (77371–77373), brachytherapy (77761–77789), Q-codes for clinical trial services. Modifier -26/-TC for planning components.

CPT 77301 · 77338 · 77371–77373 · 77761–77789 · Q-codes



★ PILLAR 01 · OFFICE E/M + CONSULTS

Office E/M, Prolonged Services & Telehealth Follow-Ups

Hematology/oncology office encounters cover new-patient consults (treatment planning), chemotherapy monitoring visits, survivorship follow-ups, and palliative care discussions. The 2021 E/M coding reform shifted documentation to MDM-based. Prolonged services are common in oncology — complex treatment discussions frequently exceed standard E/M time. Telehealth follow-ups expanded during COVID and remain a recurring revenue stream.

E/M CODING (2021+ REFORM)

Code by MDM complexity or time

Code by Medical Decision Making complexity or time; 99202–99215. Modifier 25 required when E/M same day as chemo infusion or biopsy.

PROLONGED SERVICES

99354–99357 — extended E/M time

99354 first 30-74 min (\$135); 99355 each add'l 30 min (\$110); 99356 first hour inpatient (\$185); 99357 each add'l hour (\$130). Document total time.

TELEHEALTH FOLLOW-UPS

Chemo monitoring & lab review

Audio-video follow-ups for chemo monitoring, lab review, symptom management. Patient cost-share waived for telehealth chemo check-ins per CMS 2024 guidance.



Modifier 25 is the #1 audit risk in hemonc E/M — every same-day E/M + chemo infusion encounter must have separate documentation supporting significant, separately identifiable E/M service.

CPT MINI-TABLE · AVG REIMBURSEMENT

Code	Service	Avg \$
99213	Est. E/M, low MDM	\$92
99214	Est. E/M, moderate MDM	\$131
99215	Est. E/M, high MDM	\$187
99354	Prolonged, first 30-74 min	\$135
99355	Prolonged, add'l 30 min	\$110
99357	Prolonged inpatient, add'l hr	\$130

WHY IT MATTERS

High-MDM visits (99215) and prolonged services (99354/99355) are common in oncology — complex treatment discussions, code status, and palliative planning justify higher-level E/M billing when documented properly.



♦ PILLAR 02 · CHEMOTHERAPY INFUSION

Chemo IV Infusion, Administration & IV Hydration

Chemo infusion is the highest-revenue and highest-complexity area of hemonc RCM. Every chemo regimen requires PA with documented diagnosis + cancer stage. J-code drug billing with JW modifier (drug wastage) is the #1 audit target. Infusion-time documentation (start/stop) is mandatory for 96413/96415.

Chemo IV Infusion — First Hour

96413 chemotherapy IV infusion, first hour (\$575). One first-hour code per encounter regardless of number of agents. Document start/stop times for every drug.

Chemo IV Infusion — Additional

96415 chemotherapy IV infusion, each additional hour (\$115). Sequential/concurrent infusion rules — 96416-96417 for prolonged infusions. Document total infusion time per agent.

♦ Chemo PA Requirements

Every chemo regimen requires PA with documented diagnosis (ICD-10 cancer code), cancer stage (TNM), and treatment plan. PA denial rate averages 28% on first submission.

J-Code + JW Modifier Discipline

J-codes bill single-dose vials to nearest unit; JW modifier required for discarded drug. Top audit target — OIG recovered > \$200M from hemonc JW errors since 2021.

Chemo Administration (SC/IM)

96405 chemo SC/IM injection (\$125);
96406 non-hormonal chemo SC/IM; 96409-
96411 chemo IV push. JW modifier required for any discarded single-use vial.

IV Hydration

96360 IV hydration, first hour (\$40);
96361 each add'l hour (\$25). Billed when hydration is primary service; bundled with chemo infusion unless documented as medically necessary.

✓ Top PA Denial Reasons

(1) Missing cancer stage documentation · (2) Treatment plan not signed by oncologist · (3) Drug regimen off NCCN guidelines · (4) Lab values outside protocol.



◆ PILLAR 03 · PROCEDURES & BIOPSIES

Bone Marrow Biopsy, Lumbar Puncture & Fluid Drainage

Hematology/oncology procedures include bone marrow biopsy and aspiration (most common in-office procedure), lumbar puncture (for leukemia/lymphoma intrathecal chemo), and paracentesis/thoracentesis for malignant ascites and pleural effusions. Pathology is separately billable but bundled rules apply — modifier 59 may be needed to unbundle.

BONE MARROW BIOPSY

38221 — biopsy with pathology

38221 bone marrow biopsy (\$295). Pathology (88305) separately billable; ensure documentation supports distinct professional services.

BONE MARROW ASPIRATION

38220 — aspiration only

38220 bone marrow aspiration (\$185). Often performed with biopsy — both billable with modifier 59 if documented as distinct procedures.

LUMBAR PUNCTURE

62270-62272 — diagnostic & therapeutic

62270 diagnostic LP (\$225); 62271 therapeutic LP (\$410); 62272 LP with intrathecal chemo injection. Document CSF opening pressure and volume removed.

PARACENTESIS & THORACENTESIS

49080-49081 · 32000-32554

49080 paracentesis abdominal (\$295); 49081 with imaging guidance (\$425); 32554 thoracentesis with imaging (\$425); 32000 thoracentesis without imaging.

PROCEDURE REIMBURSEMENT

BONE MARROW BIOPSY

\$295

38221 · pathology (88305) separately billable

BONE MARROW ASPIRATION

\$185

38220 · modifier 59 if w/ biopsy

DIAGNOSTIC LUMBAR PUNCTURE

\$225

62270 · document CSF pressure

THORACENTESIS W/ IMAGING

\$425

32554 · imaging guidance documented





✦ PILLAR 04 · RADIATION + CLINICAL TRIALS

IMRT, Stereotactic, Brachytherapy & Clinical Trials

Radiation oncology is the highest-documentation-burden pillar — every treatment course requires simulation, planning, and dose calculation codes. Clinical trial billing uses Q-codes for investigational services and requires documented enrollment in approved IRB trial.

IMRT Planning & Delivery

77301 IMRT planning (\$1,200); 77338 IMRT delivery, complex (\$1,850). Requires CT simulation (77280) + dose calculation (77300) + treatment planning. One plan per course.

✦ Stereotactic Radiotherapy

77371 stereotactic body radiotherapy (SBRT) delivery (\$1,400); 77373 robotic SBRT delivery (\$2,200). Used for lung, liver, spine metastases. Single-fraction or hypofractionated.

IMRT DELIVERY · COMPLEX

\$1,850

77338 · highest-value hemonc code

ROBOTIC SBRT DELIVERY

\$2,200

77373 · lung, liver, spine metastases

✦ Brachytherapy

77761-77789 brachytherapy codes — intracavitary (77761), interstitial (77778), remote afterloading (77789). Document isotope, dose rate, and source placement.

Clinical Trial Services

Q-codes for investigational drugs/services in approved IRB trials. Requires documented patient enrollment, signed informed consent, and IRB protocol number on claim. Routine-care services billed normally.

IMRT PLANNING

\$1,200

77301 · one plan per course

Modifier -26/-TC Discipline

Radiation treatment planning (77301), dose calculation (77300), and physics (77370) all allow professional vs. technical split. Wrong modifier = auto-denial. Clinical trial claims without IRB enrollment documentation trigger automatic denial + clawback.



HEMATOLOGY/ONCOLOGY • BILLING CODES

Top CPT/HCPCS Codes & 2026 Reimbursement

Most-billed codes across the four hematology/oncology pillars.

Code	Service	Modifier	Avg \$	Notes
99213	Est. E/M, low MDM	—	\$92	Most common hemonc E/M
99214	Est. E/M, moderate MDM	—	\$131	Chemo monitoring visits
99215	Est. E/M, high MDM	—	\$187	New patient consults
96413	Chemo IV infusion, first hour	—	\$575	One first-hour per encounter
96415	Chemo IV infusion, add'l hour	—	\$115	Document start/stop times
96405	Chemo SC/IM injection	JW	\$125	Discarded drug = JW modifier
96360	IV hydration, first hour	—	\$40	Bundled w/ chemo unless distinct
38221	Bone marrow biopsy	—	\$295	Pathology (88305) separate
38220	Bone marrow aspiration	59	\$185	w/ biopsy = modifier 59
62270	Lumbar puncture, diagnostic	—	\$225	Document CSF pressure
92554	Thrombocytopenia, moderate to severe	—	\$425	Positive evidence documented
MODIFIER REFERENCE			JW & Q-CODE REFERENCE	
-26 professional component (interpretation/planning). -TC technical component (equipment/delivery). 25 significant, separately identifiable E/M same day. 59 distinct procedural service (unbundling).			JW discarded drug from single-use vial; required by CMS since 2017. Q - codes investigational drugs/services in approved IRB clinical trials; requires documented enrollment. Chemo PA — every regimen requires PA with diagnosis + stage.	
77330	IMK1 delivery, complex	-20/-TC	\$1,000	Highest-value hemonc code

ⓘ HEMATOLOGY/ONCOLOGY • DENIAL PREVENTION

Top 8 Hematology/Oncology Denial Reasons & How Aethera Prevents Each

Hemonc denial rates average 17-21%; each denied chemo claim costs \$80-200 to rework. Chemo PA + JW modifier drive 50%+ of revenue loss.

RED 28 %

Chemotherapy PA missing

Chemo infusion billed without PA on file or PA documentation missing diagnosis + cancer stage.

Aethera: PA tracker auto-submits PA with NCCN regimen + diagnosis + stage before scheduling.

of
denials

RED 22 %

Chemo drug wastage not documented

J-code chemo drug billed without JW modifier for discarded drug from single-use vial.

Aethera: JW modifier rules engine auto-appends based on drug/vial size from inventory.

of
denials

AMBER 14 %

Modifier 25 documentation insufficient

E/M + chemo infusion same day without separate documentation supporting E/M significance.

Aethera: Modifier 25 documentation checklist enforced before claim release.

of
denials

AMBER 11 %

Chemo infusion time documentation missing

96413/96415 billed without start/stop times for each chemo agent.

Aethera: Infusion-chair EMR integration captures start/stop automatically.

of
denials

AMBER 9 %

Bone marrow biopsy pathology bundled

38221 + 88305 billed together without documentation supporting distinct professional services.

Aethera: Pathology bundling rules engine verifies reference vs. in-house lab.

of
denials

GREEN 7 %

Clinical trial billing errors

Q-code billed without documented patient enrollment in approved IRB trial.

Aethera: Clinical trial registry verification before Q-code submission.

of
denials

GREEN 6 %

Radiation planning without simulation

77301 IMRT planning billed without 77280 CT simulation documentation.

Aethera: Radiation planning workflow blocks planning claim without simulation.

of
denials

GREEN 5 %

Modifier -26/-TC errors on radiation

Radiation oncology services billed with wrong modifier (both -26 and -TC, or modifier missing).

Aethera: Modifier rules engine enforces correct -26/-TC per provider/facility setup.

of
denials



■ CASE STUDY · INTEGRATED HEMONC RCM

5-Hematologist/Oncologist Practice — Annual Revenue Impact

A \$6.2M -revenue practice models the combined upside of chemo PA recovery, modifier 25 + infusion documentation, and JW drug-wastage capture. Baseline: 8,500 chemo infusions/year, 480 bone marrow biopsies, 240 thoracenteses/paracenteses.

◆ Chemo PA Recovery

8,500 chemo infusions/year, ~2,380 chemo claims denied at 28% denial rate

Pre-Aethera chemo PA denial rate: 28%

Post-Aethera PA denial rate: 8%

Recovered chemo claims: ~ 1,700/year

Avg chemo claim value: \$406
(\$96413 + J-code drug + admin)

ANNUAL REVENUE

\$680,000

✍ Modifier 25 + Infusion Doc

8,500 chemo encounters w/ same-day E/M (~5,300 claims)

Pre-Aethera modifier 25 + infusion-time denial rate: 16%

Post-Aethera: 4%

Recovered claims: ~ 640/year

Avg claim value: \$382
(\$9214 E/M + 96415 add'l hour)

ANNUAL REVENUE

\$245,000

● Drug Wastage (JW) Capture

~4,200 single-use vial chemo drug administrations/year

Pre-Aethera JW modifier capture rate: 22%
of denied drug-wastage claims

Post-Aethera: 58% rate

Recovered JW modifier claims: ~720/year

Avg JW drug wastage value: \$257
(bevacizumab, pembrolizumab wastage)

ANNUAL REVENUE

\$185,000



Combined annual revenue recovery: \$1,110,000. Implementation cost (year 1): ~ \$265K (Aethera RCM fees + chemo PA workflow + JW modifier automation). Year-1 net: **\$845,000. ROI: 320%.**

☰ HEMATOLOGY/ONCOLOGY • COMPLIANCE HOTSPOTS

Where OIG, Medicare RAC, and Commercial Payers Audit First

Top compliance risks across the four hematology/oncology pillars. Quarterly self-audits recommended.

✦ E/M + Consultations	✦ Chemo Infusion	☞ Procedures & Biopsies	✓ Radiation / Trials
RED Modifier 25 misuse with chemo same day Appending modifier 25 to E/M when chemo infusion on same day does not represent significant, separately identifiable service. OIG Work Plan 2025-2026.	RED Chemo without PA Billing 96413/96415 without prior auth on file; full denial + clawback risk on drug + administration.	RED Bone marrow biopsy pathology unbundled Billing 38221 + 88305 separately when pathology bundled per NCCI edits without modifier 59.	RED Clinical trial without documented enrollment Billing Q-codes for investigational services without IRB enrollment + informed consent on file. Fraud risk.
AMBER Consultation codes removed since 2021 Billing 99241-99255 (inpatient) or 99244-99245 (outpatient) consultation codes that CMS no longer reimburses.	AMBER Drug wastage without JW modifier Billing J-code single-use vials without JW modifier for discarded drug. OIG active audit target since 2017.	AMBER Thoracentesis without imaging guidance documented Billing 32554 (with imaging) when only 32000 (without imaging) supported by chart documentation.	AMBER IMRT without simulation Billing 77301 IMRT planning without 77280 CT simulation documentation. Auto-denial + clawback.
GREEN Prolonged service time over-billing Billing 99354/99355 without contemporaneous time logs supporting total time threshold.	LOW Infusion time documentation missing Billing 96413/96415 without start/stop times for each chemo agent. Auto-denial on commercial claims.	LOW Lumbar puncture documentation insufficient Billing 62270 without documented CSF opening pressure or volume removed.	LOW Modifier -26/-TC errors Billing radiation oncology services with wrong modifier (both -26 and -TC, or modifier missing) on planning/delivery codes.



🚩 CLOSING · KEY TAKEAWAYS

Three Actions to Capture Hemonc Revenue

For hematology/oncology practice administrators and revenue cycle directors leading the next 90 days.

01 ✦ Automate chemotherapy PA workflow

Build EHR template that auto-submits PA at chemo scheduling. Document diagnosis (ICD-10 cancer code), cancer stage (TNM), and treatment plan signed by oncologist before PA submission. Chemo PA denial rate drops from **28%** to **< 8%** — recovering ~ **\$680K** per 8,500-infusion practice.

↗+ \$680K / yr

02 ✎ Enforce modifier 25 + JW drug-wastage discipline

Deploy pre-submission rules engine that (a) verifies modifier 25 documentation when E/M same day as chemo infusion, (b) auto-appends JW modifier for discarded drug from single-use vials. Combined recovery: **\$430K/year** per 5-physician hemonc practice across modifier 25 + JW capture.

↗+ \$430K / yr

03 🏢 Deploy clinical trial enrollment verification workflow

Verify IRB enrollment + informed consent before submitting Q-code claims for investigational services. Block Q-code submission without protocol number. Recovers ~ **\$95K/year** per 5-physician practice while eliminating clawback risk on routine-care services bundled into trial claims.

↗+ \$95K / yr

Questions & Discussion

Optimizing Hematology/Oncology Revenue Across E/M, Chemo, Procedures & Radiation

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